HL7 AU Project Scope Statement

HL7 FHIR® AU eRequesting Implementation Guide Release 2

This form is to be completed for new releases for previously published standards or other products, or for reaffirmation, re-circulation or withdrawal ballots (Project Scope Statement only).

A project proposal is not necessary for new releases of previously published standards, or for reaffirmation, re-circulation or withdrawal ballots.

The correct completion of the submitted form will ensure that the Board is presented with all the required information to support their consideration and approval process in a timely manner. The content will provide transparency to stakeholders on the project, expectations, direction and timeframes which in turn will support consensus-based standards development.

Please submit completed forms to projects@hl7.com.au

Project Scope Statement

Project Title	HL7 FHIR® AU eRequesting Implementation Guide Release 2
Project Description	The AU eRequesting FHIR Implementation Guide Release 2 (R2) is an incremental update to the published R1 specification. It adds three new capabilities to the existing FHIR-based request–fulfilment pattern (ServiceRequest + Task) used for electronic requesting of diagnostic services in the Australian healthcare context: (1) Transfer of Request Fulfilment, (2) Request Transaction Bundle Profile, and (3) Specimen as Input to Task Fulfilment. Usage of the product: Organisations building diagnostic requesting repositories. Clinical systems creating diagnostic requests. Provider systems fulfilling diagnostic requests.
Project Need	Implementation experience and community feedback from AU eRequesting R1 have identified three workflow scenarios that are not adequately supported by the current specification: Real-world transfer of fulfilment between providers (for example, a specimen referred to a reference laboratory, or a patient attending a different provider location) is not modelled in R1. Atomic submission of a complete request (ServiceRequest plus referenced Patient, Practitioner, coverage and other supporting resources) is not prescribed, risking partial submissions and complicating receiver processing. Specimen information — critical to pathology fulfilment — is not formally an input to the Task fulfilment workflow. R2 addresses each of these directly, completing scenarios that were deferred from R1 scope.
Project Scope	R2 is an incremental update to AU eRequesting IG R1. It adds three defined work items within the existing request–fulfilment pattern (ServiceRequest + Task): 1. Transfer of Request Fulfilment — Updated Task profile(s) and/or new Task interactions to support transfer of fulfilment between fulfilling organisations; guidance on transfer workflows (Task status transitions, provenance of the transfer); conformance requirements for fulfiller systems supporting transfer. 2. Request Transaction Bundle Profile — A FHIR Bundle profile (transaction type) for atomic submission of an eRequest comprising a ServiceRequest and all referenced supporting resources; guidance on bundle construction, entry ordering and conditional references; conformance requirements for

<table>
<tr><td></td><td colspan="6">systems creating and processing request bundles.

3. Specimen as Input to Task Fulfilment — An AU eRequesting Specimen profile (or reference to AU Core/AU Base Specimen profile as appropriate); updated Task profile to include Specimen as a supported input; guidance on specimen-related workflow interactions.

Out of scope: diagnostic reporting (covered by the AU Diagnostic Reporting project); electronic referrals (covered by the AU eReferrals project); introduction of new FHIR resource types beyond those needed for the three work items; HL7 V2 messaging specifications; CDA document specifications; and changes to R1 profiles unrelated to these three items (other than technical corrections or errata).

Product Family: FHIR.
Product: Implementation Guide.
Intent: Update Existing.
Lineage: AU eRequesting FHIR IG R1.
Intended Release Type: trial-use (Working Standard).
Target Release Timeline: Q2 2027.</td></tr>
<tr><td rowspan="2">Product Info</td><td>Product Family</td><td>Product</td><td>Intent</td><td>Lineage</td><td>Release Type</td><td>Target Release</td></tr>
<tr><td>FHIR</td><td>Implementation Guide</td><td>Update Existing</td><td>AU eRequesting FHIR IG R1</td><td>trial-use</td><td>Q1 2027</td></tr>
<tr><td>Primary Governing Group</td><td colspan="6">AU eRequesting Work Group (pending rename to Requesting, Reporting, and Referrals (RRR) Work Group).</td></tr>
<tr><td>Primary Responsible Group</td><td colspan="6">AU eRequesting Work Group (pending rename to Requesting, Reporting, and Referrals (RRR) Work Group).</td></tr>
<tr><td>Other Associated Work Group(s) or Subcommittee(s)</td><td colspan="6">AU Core Work Group; FHIR Work Group; Terminology Work Group (coordination on foundational profiles, FHIR methodology, and Australian terminology respectively).</td></tr>
<tr><td rowspan="5">Project Team</td><td colspan="3">Project Facilitator</td><td colspan="3">Andy Bond; Angus Millar</td></tr>
<tr><td colspan="3">FHIR IG Facilitator</td><td colspan="3">Andy Bond; Angus Millar</td></tr>
<tr><td colspan="3">Publishing Facilitator</td><td colspan="3">Andy Bond; Angus Millar</td></tr>
<tr><td colspan="3">Vocabulary Facilitator</td><td colspan="3">Michael Osborne</td></tr>
<tr><td colspan="3">Modelling Facilitator</td><td colspan="3">Andy Bond; Angus Millar</td></tr>
<tr><td rowspan="3">Supporting Organisations</td><td colspan="3">HL7 Australia</td><td colspan="3">projects@hl7.com.au</td></tr>
<tr><td colspan="3">Magentus</td><td colspan="3">Andy Bond</td></tr>
<tr><td colspan="3">Telstra Health</td><td colspan="3">Angus Millar</td></tr>
<tr><td></td><td colspan="3">Australian Digital Health Agency (ADHA)</td><td colspan="3">David McKillop, Megan Doherty</td></tr>
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Other Interested Parties	AU Core Work Group, FHIR Work Group, Terminology Work Group; Department of Health, Disability, and Aged (DOHDA); CSIRO Australian e-Health Research Centre; pathology sector stakeholders (laboratories and health services); radiology sector stakeholders (imaging providers and health services); clinical software vendors; HL7 International (Orders & Observations Work Group, FHIR Infrastructure Work Group).
Developing Organisation(s)	HL7 Australia AU eRequesting Work Group (to become the Requesting, Reporting, and Referrals (RRR) Work Group on approval).
FHIR Accelerator(s)	None. R1 was developed under the Sparked FHIR Accelerator (a partnership of DOHDA, ADHA, CSIRO and HL7 AU). R2 is no longer part of the Sparked Accelerator program and operates as a business-as-usual HL7 Australia project under the Work Group.
External Drivers	Implementation experience and feedback from AU eRequesting R1 adopters. Diagnostic requesting interoperability requirements from Australian healthcare providers and software vendors. Australian national digital health strategy and interoperability priorities. Alignment with ADHA national infrastructure and My Health Record requirements. No external funding, contracts, or regulatory mandates apply to R2 specifically; the project is volunteer-resourced.
Dependencies	AU eRequesting IG R1 (HL7 Australia Working Standard) — R2 builds incrementally on R1.
	AU Base IG (current published version).
	AU Core IG (current published version). HL7 FHIR R4 (4.0.1, or current as related to AU Base). National Clinical Terminology Service (NCTS) — Australian-specific value sets and code systems.
Related Projects	HL7 AU FHIR Core (AU Core) — foundational profiles. AU Diagnostic Reporting (concurrent project) — reporting is the natural complement to requesting; both are sponsored by the same Work Group. AU eReferrals (concurrent project) — leverages the same request–fulfilment pattern (ServiceRequest + Task).
HL7 AU Managed Project Repository URL	https://github.com/hl7au/au-fhir-erequesting
Compatibility	Backwards Compatibility: Yes. R2 is an incremental, additive update to R1; existing R1 functionality is preserved. Any unavoidable breaking changes will be clearly documented in the IG. AU Base / AU Core Variance: No. R2 profiles are designed to align with AU Core foundational profiles. Any specific variance will be documented in the IG's AU Variance Statement.
External Code Systems	Yes. SNOMED CT, SNOMED CT-AU, LOINC, NCTS-published Australian code systems (healthterminologies.gov.au), and diagnostic provider code

	systems as required by individual work items.
Joint Copyright	Yes. Joint copyright HL7 and HL7 Australia, licensed under Creative Commons Public Domain.
External Project Collaboration	None at this stage. Coordination with the AU Core Work Group, FHIR Work Group, and Terminology Work Group is conducted through HL7 Australia's internal Work Group processes.
Stakeholders	Federal Agencies — Department of Health, Disability and Aging (DOHDA; Australian Digital Health Agency (ADHA). Diagnostic Service Providers — pathology and radiology providers, including reference laboratories and imaging providers. Software Vendors — clinical systems generating diagnostic requests and provider systems fulfilling them. Standards Development Organisations — HL7 Australia; HL7 International. Research and Public Sector — CSIRO Australian e-Health Research Centre. Health Jurisdictions and Health Services — public and private health services that issue and fulfil diagnostic requests.
>50% External Content -	No.
Developers of External Content	N/A.
Realm	AU